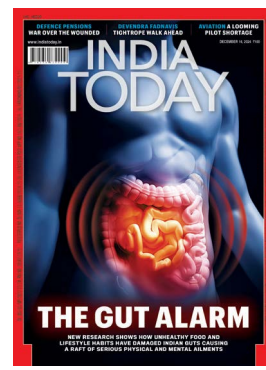
FROM THE
EDITOR-IN-CHIEF

Some things in life are truly democratic. You could be an emperor, a political heavyweight, a pontiff or a social media star. That won't put you out of harm's way with one of the most universal blights out there. But rarely do we dignify it with the attention that, say, cardiac issues, diabetes or blood pressure receive. We are talking about pruritus, or the lowly itch. More and more people are itching now. Its global prevalence rate rose 11 per cent between 1990 and 2021. The absolute number of people recorded as suffering nearly doubled to 80.7 million. In India, 42.6 per cent of a 3,000-sized sample, when asked in a survey, reported itching in the previous seven days. We are prone to treating an itch as a minor botheration, 'dashing to the derma' only if it gets too bad. But we forget that the skin is a living organ, the largest we have. Long recognised as a 'mirror of the body', abnormal appearances on it may be reflecting events unfolding deep inside. Yet, mostly we fail to read this open book.

Treating it as 'just an itch' limits our gaze. We mistake the symptom for the disease. It's an easy error to make in a country like India, where skin problems can be caused by tropical heat, humidity, synthetic clothing, pollution and overuse of cosmetics, besides stress and poor dietary habits. These can be painful, but we must distinguish between surface-level episodes and those that may be signalling a deeper problem. What calls for heightened vigil is a pattern of persistence, recurrence and spreading. Chronic pruritus may even disrupt your sleep, setting off its own negative cascade. "We are seeing more patients in whom an itch is no longer a one-off episode; it keeps returning as the underlying trigger has not gone away," says Dr Monica Bambroo, head of dermatology and cosmetology at Artemis Hospital, Gurugram.



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What could that trigger be? For some, an itch could signal an impaired liver or biliary ducts. Some may be getting warned of excess red-blood cells. In fact, everything from diabetes to thyroid or gut microbiome issues, auto-immune disorders to chronic renal dysfunction can announce themselves on the skin. These early warnings, flashed on the dermatological dashboard, can lead us to timely diagnosis and treatment of the underlying pathology. Sometimes, it may even save lives.

The good news is that new diagnostic tools can help you zero in on the exact cause and nature of an itch, opening up precisely targeted treatments. Patch or photo-patch tests can pinpoint an offending allergen in cosmetics. Multi-dimensional itch scales assess severity, duration and distribution to track pruritus arising out of liver, kidney and skin diseases. A fungal itch will now take you not to a generic ointment but a lab culture that first specifies the exact fungus, and then susceptibility tests that determine which drugs it may be sensitive to. This is important as new studies are registering drug-resistant strains. Genetic testing leads to the same path, isolating mutations linked to antifungal resistance.

Interventions, likewise, have moved beyond antihistamines to biologic drugs and enzyme inhibitors. Their names may be unpronounceable—dupilumab, deucravacitinib, difelikefalin or the Janus kinase (JAK) inhibitors class—but these are the new cures for old ailments like eczema, psoriasis, renal-origin itching, contact dermatitis and hives. They work by targeting the delicate underlying machinery: tiny inflammatory bylanes, signals passing between the immune system and nerves. "What is exciting now is that we are no longer treating every severe itch the same way," says Dr D.M. Mahajan of Apollo Hospital, Delhi.

This week, our cover story explores a subject that, like an actual itch, does not get the treatment it warrants. Diving beyond skin-deep, Senior Editor Sonali Acharjee also covers the surface extensively, with explanatory charts on the full taxonomy of itches, and what must be done about them. She points out that we all tend to lapse into self-medication, mostly with over-the-counter ointments. But using the same cream for dryness, allergy, eczema and fungal infection could make one of them worse. The fungal itch, an area of significant concern in India with its "epidemic-like" proportions, is where the quick fix can become part of the disease. Steroidal creams can bring immediate ease, but leave the fungus untreated, potentially worsening it. Besides, slathering on steroids thoughtlessly can leave behind a trail of stretch marks, loss of pigmentation or skin-thinning. But too many Indians do it: a study found the tendency to be prevalent in 50.9 per cent chronic fungal itch patients.

Passing aggravation or not, a skin condition can cause social anxiety. In one of our case studies, a woman always covers her arms in long sleeves, despite the heat, just to avoid questions about her rash. Another recalls a traumatic eczema episode before her wedding. Conversely, the skin can also suffer from too much attention: we see a fetishised approach to skincare everywhere. People are piling on products, acids, retinoids and scrubs, or blindly copying an influencer's routine. Always be aware of the many layers of surface and deep readings. Or ask a doctor. Grandma's old advice still stands too: don't scratch.

(Aroon Purie)